Patient Transaction Report

Provider: All | Date Range: Jan 1, 2025-Jan 8, 2026
Filter Charges: Service Date | Transaction Type: Associated

PATIENT NAME: Pulliam, Viktoriya | ACCOUNT #: 604899 | DOB: Dec 6, 1989

Appointment Provider Name	Claim No	Service Date	Transaction Date	Code / Desc	Balance
Ptacek, Tyler A	4609098	Jul 22, 2025	Jul 22, 2025	99204 Office Visit, New Pt., Level 4	$337.00
			Aug 14, 2025		
			Aug 14, 2025		
			Sep 9, 2025		
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$0.00
Ptacek, Tyler A	4637792	Aug 18, 2025	Aug 18, 2025	99214 Office Visit, Estab Pt., Level 4	$219.00
			Sep 18, 2025		
			Sep 18, 2025		
			Oct 23, 2025		
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$0.00
Ptacek, Tyler A	4659527	Sep 10, 2025	Sep 10, 2025	64479 INJ FORAMEN EPIDURAL C/T	$1,308.00
			Sep 10, 2025	J1100 DEXAMETHASONE PHOSPHATE 1MG	$1.80
			Oct 9, 2025		
			Oct 9, 2025		
			Oct 23, 2025		
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$0.00
Hunt, Janessa	4661285	Sep 10, 2025	Sep 10, 2025	99213 Office Visit, Estab Pat., Level 3	$150.00
			Sep 10, 2025		
			Sep 25, 2025		
			Sep 25, 2025		
			Oct 23, 2025		
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$0.00
Ptacek, Tyler A	4683518	Oct 6, 2025	Oct 6, 2025	99214 Office Visit, Estab Pt., Level 4	$219.00
			Oct 6, 2025		
			Nov 6, 2025		
			Nov 6, 2025		
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	($50.00)
Ptacek, Tyler A	4702909	Oct 23, 2025	Oct 23, 2025	62323 NJX INTERLAMINAR LMBR/SAC	$852.00
			Oct 23, 2025		

Patient Transaction Report

Provider: All | Date Range: Jan 1, 2025-Jan 8, 2026
Filter Charges: Service Date | Transaction Type: Associated

PATIENT NAME: Pulliam, Viktoriya ACCOUNT #: 604899 DOB: Dec 6, 1989

Appointment Provider Name	Claim No.	Service Date	Transaction Date	Code / Desc	Balance
Ptacek, Tyler A	4702909	Oct 23, 2025	Nov 20, 2025	[illegible]	
			Nov 20, 2025	A	
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	($20.00)
Hougland, Claudia	4705596	Oct 24, 2025	Oct 24, 2025	99214 Office Visit, Estab Pt, Level 4	$219.00
			Nov 6, 2025	[illegible]	
			Nov 6, 2025	[illegible]	
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$0.00
Hougland, Claudia	4705666	Oct 24, 2025	Oct 24, 2025	36415 VENIPUNCT, ROUTINE*	$15.00
			Oct 24, 2025	80048 BASIC METABOLIC PANEL	$38.00
			Oct 24, 2025	85025 COMPLETE CBC W/AUTO DIFF WBC	$18.00
			Nov 6, 2025	[illegible]	
			Nov 6, 2025	[illegible]	
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$0.00
Ptacek, Tyler A	4736094	Nov 25, 2025	Nov 25, 2025	61070 BRAIN CANAL SHUNT PROCEDURE	$600.00
			Nov 25, 2025	62350 IMPLANT SPINAL CANAL CATH	$1,254.00
			Nov 25, 2025	62362 IMPLANT SPINE INFUSION PUMP	$4,254.00
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$6,108.00
Ptacek, Tyler A	4742962	Dec 1, 2025	Dec 1, 2025	62273 TREAT EPIDURAL SPINE LESION	$1,160.00
			Dec 1, 2025	77003 FLUOROGUIDE FOR SPINE INJECT	$478.73
			Jan 2, 2026		
			Jan 2, 2026	[illegible]	
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$25.00
Ptacek, Tyler A	4743363	Dec 4, 2025	Dec 4, 2025	62370 ANL SP INF PMP W/MDREPRG&FIL	$400.00
			Dec 4, 2025	99024 Follow-up visit (Global)	$0.00
			Jan 2, 2026	[illegible]	
			Jan 2, 2026	[illegible]	

Patient Transaction Report

Provider: All
Filter Charges: Service Date
Date Range: Jan 1, 2025-Jan 8, 2026
Transaction Type: Associated

PATIENT NAME: Pulliam, Viktoriya ACCOUNT #: 604899 DOB: Dec 6, 1989

Appointment Provider Name	Claim No	Service Date	Transaction Date	Code / Desc	Balance
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$25.00
Ptacek, Tyler A	4769804	Jan 5, 2026	Jan 5, 2026	62368 ANALYZE SPINE INFUSION PUMP	$413.00
CLAIM BALANCE			CLAIM BALANCE	CLAIM BALANCE	$413.00
ACCOUNT SUMMARY					[illegible]
Charge					$11,936.53
Contractual					
Payment					
Total Balance					

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 07/22/2025

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

07/22/2025 **Progress Notes: Tyler Ptacek MD**

Chief Complaint

1. PM had prior infusion in neck on going pain. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
2. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
3. Left arm is spasming with placing arm behind back
4. Feels "twitching" throughout entire body
5. Seen numerous neurologists
6. Taking numerous OTCs

History of Present Illness

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Current Medications

Taking

- Advil 200 MG Tablet 1 tablet with food or milk as needed Orally Three times a day
- Lyrica 50 MG Capsule 1 capsule Orally twice a day
- Tylenol 325 MG Tablet 1 tablet as needed Orally every 6 hrs

Medication List reviewed and reconciled with the patient

Medical History

- Medical History Verified.

Surgical History/Major Diagnostic Procedure

- cervical fusion 2021
- hysterectomy 2023

Family History

- No Family History documented.

Social History

ETOH: socially
smoke: none.

Allergies

- Penicillin

Hospitalization

- No Hospitalization History.

Vital Signs

BP: **133/89**, HR: **89**, Ht: 65, Wt: **155**, BMI: **25.79**, Oxygen sat %: **98**, Ht-cm: 165.1 cm, Wt-kg: 70.31 kg.

Examination

Pain Exam:

General Mild to moderate distress due to pain, Respiratory drive normal, Alert and oriented x 3, No gross motor deficits, Slow ambulation due to pain.
Trigger points taut bands of muscles with clear trigger points that replicate pain with pressure and restrict range of motion at the bilateral cervical paraspinals and trapezius

head and neck region shows limited movement and resistance to stretching muscles that is producing an abnormal posture. The affected muscles are: splenius capitis, sternocleidomastoid, scalene complex, levator scapulae, trapezius

SHe has rigidity left upper extremity.
Cervical radiculitis left Severe guarding of the neck
TTP at the cervical paraspinal and trapezius musculature
Decreased ROM due to severe pain
Spurling's positive left
Decreased sensation pin prick and light touch on the left C6 (thumb and index finger)
Reflexes 2/4 and symmetrical UE's - Exception of decreased reflexes at the left brachioradialis
Normal strength testing of the UE's exception 4/5 at the left wrist extensors.

Assessments

1. Failed neck syndrome - M96.1 (Primary)
2. Cervical radiculopathy - M54.12
3. Cervical cord myelomalacia - G95.89
4. Cervical dystonia - G24.3
5. Motor vehicle traffic accident, sequela - V89.2XXS

Treatment

1. Failed neck syndrome

Notes: Patient underwent C5-C6-C7 fusion in September 2019 following car accidents. Initial post-surgical relief was noted, but symptoms worsened within a year. Current symptoms include muscle spasms, twitches, and pain, particularly in the arm, mid-back, and neck. Patient reports taking high doses of NSAIDs (Advil 3200 mg daily) and acetaminophen (4000 mg daily) for two months, along with Lyrica, without adequate pain control. Vicodin was prescribed by Dr. Schubert for severe pain. Recent MRI shows worsening findings, including new OPLL and moderate stenosis. Patient reports symptom exacerbation following a recent flu illness.
Plan:
- Order CT myelogram at Campbell County on July 29th at 8:00 AM to further evaluate spinal pathology
- Trial of baclofen for muscle spasms and pain
- Consider Butrans patch (buprenorphine) for pain management
- Evaluate candidacy for spinal cord stimulator based on imaging results
- Review upcoming CT myelogram results to determine best course of action
- Provide detailed clinical note for patient's legal team to define future medical needs
- Follow up after CT myelogram to discuss results and treatment plan

2. Cervical radiculopathy

Notes: imaging being updated, has MODERATE CCS due to opll s/p instrumentation at this same level

3. Cervical cord myelomalacia

Notes: due to MVA

4. Cervical dystonia

Start Baclofen Tablet, 10 MG, 1 tablet as needed, Orally, TID, 10 days, 30 Tablet, Refills 0

Notes: I will prescribe Baclofen to aid in muscle relaxation, as well as for it's neuropathic component. Discussed the common side effects which include drowsiness, sleepiness, and dizziness. The patient voiced understanding.

Discussed botox

5. Motor vehicle traffic accident, sequela

Notes: reguarding her MVA:
this has resulted in cervical myelomalacia - requiring cervical spinal fusion

She may need further surgical intervention in the form of ADCF for adjacent segment disease

We have discussed SCS, pain pump trial

We have discussed ongoing medications, pt, injections

She will likely need updated imaging every 3 years for monitoring

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 10/23/2025 at 10:07 AM MDT
Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 07/22/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

08/18/2025 **Tyler Ptacek MD**

Chief Complaint

1. Medication Recheck - My pain has been so much better, it is almost eliminated but it hurts if I over do it. I do have some tingling. I am having congestion. Cutting the lyrica in half has assisted with this. With cutting the pill in half it seems that some of the pain is trying to come back though.
2. had prior infusion in neck on going pain Galloway, Linda, M. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
3. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
4. Left arm is spasming with placing arm behind back
5. Feels "twitching" throughout entire body
6. Seen numerous neurologists
7. Taking numerous OTCs

History of Present Illness

Disability indexing:

Disability scale PDI =21 Date of administration:8/18/25.

Follow up:

Since the last visit Viktoriya Pulliam presents for follow-up of chronic pain management. She reports significant improvement with baclofen, describing it as "fantastic" and providing "almost complete relief" at the initial full dose. However, she experienced severe congestion as a side effect, especially in combination with Lyrica, to the point where she "couldn't even swallow water." Reducing the baclofen dose to half and using Mucinex has helped manage this side effect.

The patient notes that her pain is starting to return with the reduced baclofen dose. She reports feeling "tingliness" and experiencing pain when carrying a 5-pound object while helping her husband. She also mentions getting excited and cleaning the house after initial relief, which resulted in increased pain.

Regarding Lyrica, the patient reports significant side effects including dry mouth, "loopiness," and missed appointments. She expresses concern about these cognitive effects, stating, "It's terrible. I miss things." Lyrica has been helpful for sleep and relieving muscle twitching, but the daytime side effects are problematic.

The patient discusses her diagnosis of OPLL (Ossification of the Posterior Longitudinal Ligament), describing it as the ligament being "calcified and growing towards your spinal cord." She is considering seeking a third opinion from spine surgeons at Black Hills and has an upcoming appointment with Dr. Woodruff.

In terms of treatment adherence, the patient has been taking Lyrica twice daily (midday and night) and is now considering taking it only at night to minimize daytime side effects. She is also adjusting her baclofen dosage, currently taking half the original dose.

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described

as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Current Medications

Taking

- Advil 200 MG Tablet 1 tablet with food or milk as needed Orally Three times a day
- Baclofen 10 MG Tablet 1/2 tablet as needed Orally TID , stop date 09/30/2025
- Lyrica 50 MG Capsule 1 capsule Orally twice a day
- Tylenol 325 MG Tablet 1 tablet as needed Orally every 6 hrs

Not-Taking/PRN

- Butrans 5 MCG/HR Patch Weekly 1 patch to skin Transdermal

Medication List reviewed and reconciled with the patient

Medical History

- Medical History Verified.

Surgical History/Major Diagnostic Procedure

- cervical fusion 2021
- hysterectomy 2023

Family History

- No Family History documented.

Allergies

- Penicillin

Hospitalization

- No Hospitalization History.

tal Signs

Examination

Pain Exam:

General Mild to moderate distress due to pain, Respiratory drive normal, Alert and oriented x 3, No gross motor deficits, Slow ambulation due to pain.
Trigger points taut bands of muscles with clear trigger points that replicate pain with pressure and restrict range of motion at the bilateral cervical paraspinals and trapezius

head and neck region shows limited movement and resistance to stretching muscles that is producing an abnormal posture. The affected muscles are: splenius capitis, sternocleidomastoid, scalene complex, levator scapulae, trapezius

SHe has rigidity left upper extremity.
Cervical radiculitis left Severe guarding of the neck
TTP at the cervical paraspinal and trapezius musculature
Decreased ROM due to severe pain
Spurling's positive left
Decreased sensation pin prick and light touch on the left C6 (thumb and index finger)
Reflexes 2/4 and symmetrical UE's - Exception of decreased reflexes at the left brachioradialis
Normal strength testing of the UE's exception 4/5 at the left wrist extensors.

Assessments

1. Cervical dystonia - G24.3 (Primary)
2. Cervical radiculopathy - M54.12
3. Chronic pain - G89.29

Treatment

1. **Cervical dystonia**

Notes: baclofen
decrease lyrica - to just night

2. Cervical radiculopathy

Notes: Patient reports significant improvement with baclofen, describing it as "fantastic" and providing "almost complete relief" at the initial full dose. However, she experienced increased congestion as a side effect, especially in combination with Lyrica. Reducing the baclofen dose to half improved the congestion but resulted in the return of symptoms, including tingling sensations and pain with light activities. The patient also reports significant side effects from Lyrica, including loopiness, missed appointments, and dry mouth. The underlying cause of the symptoms may be related to the patient's OPLL, with the calcified ligament growing towards the spinal cord.
Plan:
- Adjust Lyrica regimen: decrease to 50 mg PO at bedtime only, discontinue daytime dose
- Modify baclofen regimen: continue at half dose, patient instructed to increase to full dose if pain returns after Lyrica reduction
- Refer to Dr. Woodruff for third opinion on OPLL management with appointment scheduled for Wednesday in July
- Order Neuropsychological evaluation for potential future implantable device consideration
- Follow-up after Dr. Woodruff's appointment to discuss surgical options or consider intrathecal baclofen trial
- If intrathecal baclofen trial is pursued, coordinate with physical therapy for modified Ashworth scale assessment 4 hours post-trial
- Continue current physical therapy regimen

Referral To:
Reason:Please call the patient to schedule, thank you! | Please eval for candidacy for implantable therapy for pain. Is there a psychological contraindication?

3. Chronic pain

Referral To:
Reason:Please call the patient to schedule, thank you! | Please eval for candidacy for implantable therapy for pain. Is there a psychological contraindication?

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 08/21/2025 at 07:30 AM MDT
Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 08/18/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 09/10/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

09/10/2025 **Janessa Hunt, CNP**

Chief Complaint

1. PM Next steps and possible injection. I would like to talk about going back up on my lyrica. Dr. Woodruff wants the injection prior to proceeding baclofen trial. I didn't take my baclofen this morning to gauge the injection reaction
2. had prior infusion in neck on going pain. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
3. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
4. Left arm is spasming with placing arm behind back
5. Feels "twitching" throughout entire body
6. Seen numerous neurologists
7. Taking numerous OTCs

History of Present Illness

Disability indexing:

Disability scale PDI =26 Date of administration:9/10/25.

Follow up:

Since the last visit referred for neuropsych eval not completed
Consult wtih Dr. Woodruff

The patient reports ongoing neck pain that radiates to her left arm and legs. She describes the pain in her legs as feeling like the flu, with achiness and cramping. She notes that her left arm experiences spasms. She mentions that her symptoms worsen when carrying her daughter, requiring her to take Vicodin for relief during these particularly bad times.

She is currently taking Lyrica and baclofen to manage her symptoms. She reports that these medications make her pain and spasticity tolerable on a day-to-day basis. However, she is experiencing significant side effects from these medications. Lyrica or baclofen causes dry mouth and inflamed sinuses. Attempts to lower the Lyrica dose have not significantly improved these side effects, and she experiences rebound effects. Baclofen causes frequent urination, occurring every 15 minutes, which she finds particularly inconvenient when traveling for the first couple of hours after taking. She feels aside from side effects the medication regimen is helpful for her pain and spasticity

The patient expresses concern about the potential need for fusion surgery, particularly at the C4-5 level. She is hesitant about pursuing fusion, fearing a "domino effect" where additional fusions may be needed in the future due to increased pressure on other vertebrae. She states she would prefer to explore other options, such as a pain pump, before considering fusion surgery-pending results of recommended injection by Dr. Woodruff. She has previous 2 level cervical fusion

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative

treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Current Medications

Taking

- Advil 200 MG Tablet 1 tablet with food or milk as needed Orally Three times a day
- Baclofen 10 MG Tablet 1/2 tablet as needed Orally TID , stop date 09/30/2025
- Tylenol 325 MG Tablet 1 tablet as needed Orally every 6 hrs
- Lyrica 50 MG Capsule 1 capsule Orally twice a day

Not-Taking/PRN

- Butrans 5 MCG/HR Patch Weekly 1 patch to skin Transdermal

Medication List reviewed and reconciled with the patient

Medical History

- Medical History Verified.

Surgical History/Major Diagnostic Procedure

- cervical fusion 2021
- hysterectomy 2023

Family History

- No Family History documented.

Social History

ETOH: socially
smoke: none.

Allergies

- Penicillin

Hospitalization

- No Hospitalization History.

Vital Signs

BP: **118/80**, HR: **84**, Ht: 65, Oxygen sat %: **98**, Ht-cm: 165.1 cm.
pain 3/10.

Examination

Pain Exam:

General Mild to moderate distress due to pain, Respiratory drive normal, Alert and oriented x 3, No gross motor deficits, Slow ambulation due to pain.
Trigger points taut bands of muscles with clear trigger points that replicate pain with pressure and restrict range of motion at the bilateral cervical paraspinals and trapezius

head and neck region shows limited movement and resistance to stretching muscles that is producing an abnormal posture. The affected muscles are: splenius capitis, sternocleidomastoid, scalene complex, levator scapulae, trapezius

SHe has rigidity left upper extremity.
Cervical radiculitis left Severe guarding of the neck
TTP at the cervical paraspinal and trapezius musculature
Decreased ROM due to severe pain
Spurling's positive left
Decreased sensation pin prick and light touch on the left C6 (thumb and index finger)
Reflexes 2/4 and symmetrical UE's - Exception of decreased reflexes at the left brachioradialis
Normal strength testing of the UE's exception 4/5 at the left wrist extensors.

Assessments

1. Cervical dystonia - G24.3 (Primary)
2. Cervical radiculopathy - M54.12
3. Chronic pain - G89.29

Treatment

1. Cervical dystonia

Notes: Continues baclofen and decreased Lyrica at HS only

2. Cervical radiculopathy

Notes: Dr. Woodruff note - "My recommendation at this point is bilateral C4-5 transforaminal epidural steroid injections with Dr. Ptacek. She can call me 7 to 10 days later to let me know how it helps. If she gets significant relief with these injections but they are short-lived then we could possibly consider a fusion at the C4-5 level. If she gets no relief then I think we would have to assume that a lot of her symptoms are due to her spinal cord myelomalacia and problems related to her initial injury. At that point, I think that the pain pump could be an option."

We discussed options such as further conservative cares: physician directed HEP, PT, injections, surgical referral

I will order a cervical transforaminal epidural steroid injection for the patient's cervical radicular pain. The risks and benefits of the procedure were discussed in detail with the patient (worsening pain, reaction to medication, infection, injury to nerve, paralysis and even death). The level(s) that will be ordered include: Bil C4/5

Plan:
- Proceed with bilateral C4-C5 transforaminal epidural injection under fluoroscopy guidance
- Continue Lyrica at night
- Use baclofen as needed
- Follow up 2-4 weeks post-injection
- Call Dr. Woodruff 7-10 days post-injection to report effectiveness
-Pending results she would like to consider baclofen trial

JHunt, CNP

Electronically signed by Janessa Hunt , CNP on 10/28/2025 at 08:02 AM MDT
Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Janessa Hunt, CNP 09/10/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 09/10/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

09/10/2025 **Tyler Ptacek MD**

Chief Complaint

1. Here for Cervical Transforaminal epidural injection
2. had prior infusion in neck on going pain. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
3. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
4. Left arm is spasming with placing arm behind back
5. Feels "twitching" throughout entire body
6. Seen numerous neurologists
7. Taking numerous OTCs

History of Present Illness

Disability indexing:

Disability scale PDI =26 Date of administration:9/10/25.

Injection:

The patient is here for a procedure for severe pain control Epidural for radiating pain. Feels well except for the pain. Denies F/C/N/V/D. Patient continues with severe neck pain radiating to the arm. Pain is severe >7/10 and debilitating - affecting ADL's and sleep. The patient continued conservative cares including self-directed HEP, Stretching, mindfulness, and physician directed HEP for the last 6+ weeks without meaningful relief of the severe pain.

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Vital Signs

Examination

Pain Exam:

General Mild to moderate distress due to pain, Respiratory drive normal, Alert and oriented x 3, No gross motor deficits, Slow ambulation due to pain.
Trigger points taut bands of muscles with clear trigger points that replicate pain with pressure and restrict range of motion at the bilateral cervical paraspinals and trapezius

head and neck region shows limited movement and resistance to stretching muscles that is producing an abnormal posture. The affected muscles are: splenius capitis, sternocleidomastoid, scalene complex, levator scapulae, trapezius

SHe has rigidity left upper extremity.
Cervical radiculitis bilateral Severe guarding of the neck
TTP at the cervical paraspinal and trapezius musculature
Decreased ROM due to severe pain
Spurling's positive bilaterally
Reflexes 2/4 and symmetrical UE's
Normal strength testing of the UE's exception at the bilateral traps.

Assessments

1. Cervical spondylosis with radiculopathy - M47.22 (Primary)
2. Cervical spinal stenosis - M48.02
3. Cervicalgia - M54.2
4. Chronic pain syndrome - G89.4

Procedures

Cervical transforaminal epidural steroid injection (TESI):

Pre/Post operative diagnosis Cervical radiculitis, neck pain.
Surgeon Tyler A. Ptacek, MD.
Indications The patient has radicular pain that has resulted in a functional impairment for at least 6 weeks. Pain is at least 3/10 and interferes with at least 2 ADL's. There is advanced imaging with nerve root compression/stenosis that correlates with clinical findings.
Procedure After informed consent of the potential risks: bleeding/infection/injury of neurovascular structures/reaction to medication, the benefits, and the alternatives, the patient was placed in a lateral position. Sterile prep and drape were completed. Under fluoroscopic imaging, the LEFT C4/5 level was identified from above and below. A 3.5 inch x 25g spinal needle was then advanced using fluoroscopic guidance as well as tactile sensation until the needle was ascertained to be in the lateral foramen. After negative aspiration, iodinated contrast, omnipaque 240 was then injected and needle placement confirmed with fluoroscopic imaging. A total of 0.5cc of contrast was injected. Subsequently, after negative aspiration, 5mg Decadron PF mixed with 0.5cc 1% Lidocaine was injected. The needle was withdrawn and sterile dressing applied.

Next under fluoroscopic imaging, the RIGHT C4/5 level was identified from above and below. A 3.5 inch x 25g spinal needle was then advanced using fluoroscopic guidance as well as tactile sensation until the needle was ascertained to be in the lateral foramen. After negative aspiration, iodinated contrast, omnipaque 240 was then injected and needle placement confirmed with fluoroscopic imaging. A total of 0.5cc of contrast was injected. Subsequently, after negative aspiration, 5mg Decadron PF mixed with 0.5cc 1% Lidocaine was injected. The needle was withdrawn and sterile dressing applied.

The patient was taken to the recovery room for the usual recovery time prior to discharge with written and verbal instructions per routine. Fluoro images were permanently saved for future reference.
Pre-procedure pain score 3.
Post-procedure pain score 2
Percentage relief: 10.

Procedure Codes

- 64479 INJ FORAMEN EPIDURAL C/T, Modifiers: 50
- Q9966 LOCM 200-299MG/ML IODINE,1ML
- J1100 DEXAMETHASONE PHOSPHATE 1MG, Units: 10.00

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 10/23/2025 at 10:06 AM MDT

Sign off status: Completed

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 09/10/2025

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 09/10/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/06/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

10/06/2025 **Tyler Ptacek MD**

Chief Complaint

1. PM discuss pain pump req ONLY Ptacek. I feel like the last injection made everything worst. I am doing lyrica BID and would like to talk about a dose readjustment. I am pretty much over everything and would really like to move forward with pain pump
2. had prior infusion in neck on going pain. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
3. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
4. Left arm is spasming with placing arm behind back
5. Feels "twitching" throughout entire body
6. Seen numerous neurologists
7. Taking numerous OTCs

History of Present Illness

Disability indexing:

Disability scale PDI =26 Date of administration:9/10/25.

Follow up:

Since the last visit Viktoriya Pulliam is 35 y/o femail with a history of cervical fusion (C5-C7) and spinal cord injury following two car accidents in 2019 and 2020, presenting for follow-up of chronic neck pain, spasticity, and recent migraines. She has been diagnosed with myelomalacia and ossification of the posterior longitudinal ligament (OPLL).

The patient reports that her pain, previously concentrated in her head and neck with arm spasms, has recently shifted more towards her shoulder blades. She notes increased difficulty sleeping and persistent stuffiness, which she attributes to her medications (Lyrica or baclofen). After a recent epidural injection and temporary medication cessation, she experienced clear breathing for seven days, but symptoms returned upon restarting her medications. She is currently back on her previous dosage of baclofen twice daily, as once nightly was insufficient.

Viktoriya expresses concern about needing pain medication in addition to baclofen and is considering a baclofen pump for spasticity and pain management. She reports constant feelings of tightness, soreness, and tenderness, with a frequent urge to "pop" her neck due to pressure. Chiropractic adjustments have not provided relief. She attributes recent migraines to increased stress from a new job, reporting two episodes in the past week. Rizatriptan has been helpful for migraine management, but she has exhausted her supply.

The patient discontinued Butrans (buprenorphine transdermal) and has been supplementing her pain management with over-the-counter Advil and Tylenol. She expresses a desire to increase her activity level without fear of exacerbating her symptoms. Viktoriya reports that her neck and arm pain due to spasticity is profound and noticeable to others.

Injection:

Following up after 9/10/25 bilateral C4/5 TFE - 20% relief.

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better

with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Current Medications

Taking

- Advil 200 MG Tablet 1 tablet with food or milk as needed Orally Three times a day
- Lyrica 50 MG Capsule 1 capsule Orally Twice a day
- Tylenol 325 MG Tablet 1 tablet as needed Orally every 6 hrs

Not-Taking/PRN

- Butrans 5 MCG/HR Patch Weekly 1 patch to skin Transdermal

Medication List reviewed and reconciled with the patient

Medical History

- Medical History Verified.

Surgical History/Major Diagnostic Procedure

- cervical fusion 2021
- hysterectomy 2023

Family History

- No Family History documented.

Social History

ETOH: socially
smoke: none.

Allergies

- Penicillin

Hospitalization

- No Hospitalization History.

Vital Signs

Examination

Pain Exam:

General Mild to moderate distress due to pain, Respiratory drive normal, Alert and oriented x 3, No gross motor deficits, Slow ambulation due to pain.
Trigger points taut bands of muscles with clear trigger points that replicate pain with pressure and restrict range of motion at the bilateral cervical paraspinals and trapezius

head and neck region shows limited movement and resistance to stretching muscles that is producing an abnormal posture. The affected muscles are: splenius capitis, sternocleidomastoid, scalene complex, levator scapulae, trapezius

SHe has rigidity left upper extremity.
Cervical radiculitis bilateral Severe guarding of the neck
TTP at the cervical paraspinal and trapezius musculature
Decreased ROM due to severe pain
Spurling's positive bilaterally
Reflexes 2/4 and symmetrical UE's
Normal strength testing of the UE's exception at the bilateral traps.

Assessments

1. Spinal cord injury, C5-C7 - S14.105A (Primary)
2. Hemiparesis, unspecified hemiparesis etiology, unspecified laterality - G81.90
3. Myelomalacia of cervical cord - G95.89
4. Spasticity - R25.2
5. Chronic pain - G89.29
6. Migraines - G43.909

Treatment

1. Spinal cord injury, C5-C7

Start Pregabalin Capsule, 75 MG, 1 capsule, Orally, bid, 30 days, 60 Capsule, Refills 1
PROCEDURE: Pump Trial (62323) (Ordered for 10/06/2025) (Performed Date - 10/09/2025)

2. Hemiparesis, unspecified hemiparesis etiology, unspecified laterality

Stop Butrans Patch Weekly, 5 MCG/HR, 1 patch to skin, Transdermal
Stop Lyrica Capsule, 50 MG, 1 capsule, Orally, Twice a day
Refill Baclofen Tablet, 10 MG, 1 tablet as needed, Orally, TID, 90 days, 270 Tablet, Refills 1
LAB: CBC Auto Diff (Ordered for 10/06/2025)
LAB: BMP (Basic Metabolic Panel) (Ordered for 10/06/2025)
LAB: PLATELET FUNCTION TEST (Ordered for 10/06/2025)
LAB: MRSA BY NASAL SCREEN, NAT (SANFORD) (Ordered for 10/06/2025)
IMAGING: MRI Lumbar without contrast (72148) (Ordered for 10/06/2025) (Performed Date - 10/16/2025)
PROCEDURE: Pump Trial (62323) (Ordered for 10/06/2025) (Performed Date - 10/09/2025)

Notes: I will order an intrathecal drug delivery trial to be performed per standard protocol. This will be performed as the patient has failed several other conservative therapies and has intractable pain that interferes with activities of daily living

Referral To:
Reason:Please call the patient to schedule, thank you! | modified ashworth evaluation + PT 2-3x a week for 6-8 weeks please work on stabilization and strengthening

Referral To:
Reason:Please call the patient to schedule, thank you! | Please eval for candidacy for implantable therapy for pain. Is there a psychological contraindication?

3. Myelomalacia of cervical cord

PROCEDURE: Pump Trial (62323) (Ordered for 10/06/2025) (Performed Date - 10/09/2025)

Notes: Patient has a history of C5-C7 fusion and spinal cord injury following two car accidents in September 2019 and February 2020. She experiences significant spasticity, which responds well to baclofen. The patient also has ossification of the posterior longitudinal ligament (OPLL), contributing to spinal cord compression. Three surgeons have agreed that no further cervical surgery is currently indicated, though future fusion at C4 or C5 may be necessary. Recent epidural injections provided minimal relief (20-30% symptom improvement) and temporarily worsened symptoms when medications were weaned.
Plan:
- Continue baclofen
- Continue Lyrica, increase to 75 mg PO BID
- Arrange modified Ashworth scale assessment to evaluate range of motion and spasticity levels
- Schedule intrathecal baclofen pump trial
- Order lumbar spine MRI to assess catheter placement feasibility
- Refer for psychiatric evaluation with Advantage Point
- If trial successful and evaluation completed, proceed with intrathecal baclofen pump implantation
- Annual monitoring of cervical spine condition as recommended by Dr. Woodruff

4. Spasticity

PROCEDURE: Pump Trial (62323) (Ordered for 10/06/2025) (Performed Date - 10/09/2025)

5. Chronic pain

Notes: Patient reports ongoing neck pain and more recent onset of shoulder blade pain. Sleep has become more difficult due to pain. Previous chiropractic adjustments were ineffective. The pain is likely multifactorial, stemming from the spinal cord injury, cervical fusion, and muscle tension related to spasticity.
Plan:
- Continue Lyrica, increase to 75 mg PO BID for nerve pain management
- Reassess pain control after intrathecal baclofen pump trial and potential implantation
- Consider adding pain medication to intrathecal pump if baclofen alone is insufficient

6. Migraines

Notes: Patient reports recent onset of migraines, possibly related to new job stress. She has experienced two migraines in the past week. Rizatriptan has been helpful but supply is depleted. Previous trial of sumatriptan caused chest discomfort.
Plan:
- Provide samples of gepant medication for acute migraine treatment

- Have Jordan order gepant medication prescription
- Educate patient on gepant mechanism of action and potential benefits

7. Others

Notes: Patient reports chronic nasal congestion, which she attributes to possible side effects of Lyrica or baclofen. Symptoms improved temporarily when medications were discontinued but returned upon restarting.
Plan:
- Monitor nasal congestion symptoms
- Reassess potential medication side effects at follow-up visits

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 10/23/2025 at 10:05 AM MDT
Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 10/06/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/23/2025

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

10/23/2025 **Tyler Ptacek MD**

Chief Complaint

1. Here for Intrathecal Drug Delivery Trial for severe spasticity
2. Failed 12 months of conservative cares
3. Severe, chronic, debilitating pain
4. No further surgery indicated
5. Passed Neuropsych eval
6. I feel like the last injection made everything worst. I am doing lyrica BID and would like to talk about a dose readjustment. I am pretty much over everything and would really like to move forward with pain pump
7. had prior infusion in neck on going pain. I had a C5/6 6/7 fusion, this helped for the first year but things have gradually gotten worse. Turning my head is rough. In order for me to function during the day I have to take max doses of tylenol and advil. I started taking lyrica to help sleep but it does not help. At this point, I am looking at a pain pump as an option.
8. car accident sept 2019, then feb 2020 - need cervical fusion C5/6 and C6/7 by Dr Schubert for myelomalacia - helped symptoms for 1 year
9. Left arm is spasming with placing arm behind back
10. Feels "twitching" throughout entire body
11. Seen numerous neurologists
12. Taking numerous OTCs

History of Present Illness

Disability indexing:

Disability scale PDI =26 Date of administration:9/10/25.

Pain pump:

Patient is here for: a pump trial for severe and intractable as well as chronic spasticity due to spinal cord injury. Feeling well today besides pain. Denies F/C/N/V/D. No recent infections. Denies blood thinners. Labs reviewed and there are no contraindications to neuraxial procedures. Neuropsychological clearance was reviewed and the patient has no contraindication. There have been no active substance abuse issues identified. There is no surgical procedure that is indicated to help with the patient's pain. We have extensively discussed the risks/benefits/alternatives to this procedure .

Injection:

Response to previous injection 9/10/25 bilateral C4/5 TFE - 20% relief.

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Vital Signs

Examination

Pain Exam:

General

Patient alert and oriented 3. Speech fluent. No acute distress noted. Posture upright but with mild increased tone noted in dependent limbs. Gait assisted.

Cranial Nerves:
Cranial nerves II-XII grossly intact. No facial asymmetry, dysarthria, or tongue deviation. Pupils equal, round, reactive to light. Extraocular movements intact.

Motor Examination:
Right upper extremity shows increased tone with mild flexor posturing at the elbow and wrist. Strength 4+/5 proximally and 4-/5 distally.
Left upper extremity tone normal, strength 5/5.
Both lower extremities show increased tone, greater on the right. Hip flexion 4/5, knee extension 4-/5, plantar flexion 4-/5 on right; left leg strength 4+/5 throughout.

Tone:
Spasticity present in the right upper extremity and both lower extremities.
Modified Ashworth Scale:

- Right upper extremity: Elbow 2, wrist 2, fingers 1+.
- Right lower extremity: Hip flexors 2, knee extensors 2, ankle plantarflexors 1+.
- Left lower extremity: Hip flexors 1+, knee extensors 2, ankle plantarflexors 1+.
  Catch noted on rapid passive movement, more pronounced with increased velocity. No clonus in upper extremities; 2-3 beat clonus at bilateral ankles.

Reflexes:
Biceps 3+ right, 2+ left.
Triceps 3+ right, 2+ left.
Brachioradialis 3+ right, 2+ left.
Patellar 4+ bilaterally.
Achilles 3+ bilaterally.
Neg clonus

.

Assessments

1. C5-C7 incomplete quadriplegia - G82.54
2. Dystonia - G24.9
3. Chronic pain syndrome - G89.4

Procedures

Intrathecal Drug Delivery Trial:
Pre/Post operative diagnosis C5-7 cervical myelopathy and spascity dystonia sequela.
Surgeon Tyler A. Ptacek.
Indications severe spasticity s/p spinal cord injury, inefficacy with oral baclofen and having side effects. No surgical indications have been identified which would be reasonably expected to relieve pain. No pre-procedure contraindications were identified with psychological and physiologic screening.
Informed Consent The risks, benefits, potential complications (bleeding or spinal headache) and alternatives were discussed with the patient as per the consent form and informed consent was obtained. A time-out was taken prior to the start of the procedure to confirm the patient, the procedure, and the site to be correct.
Procedure The patient was placed in the prone position. Sterile prep and drape were completed. Under fluoroscopic imaging, the lumbar levels were identified from above and below. Local anesthetic infiltration was performed to anesthetize the needle track. Using fluoroscopic guidance, the intrathecal space was entered at the L2-3 level with a single pass using a 3.5 inch x 25g spinal needle through an 18g 1.5 hypodermic needle. Clear CSF was noted. Intrathecal placement was confirmed using Omnipaque 240 contrast, revealing intrathecal flow and further confirmed with AP and lateral fluoro views. Subsequently, preservative free Baclofen 50mcg was injected without pain or paresthesia. The needle was then withdrawn and sterile dressing applied. The patient was observed for the usual post procedure recovery time to evaluate for pain relief and potential side effects or complications. Please see nursing notes for further details. The patient was discharged in the care of a driver with instructions per routine. Fluoro images were permanently saved for future reference.
Pre-procedure pain score 3.
Post-procedure pain score 0
Percentage relief: 100.

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/23/2025

ocedure Codes

- 62323 NJX INTERLAMINAR LMBR/SAC
- Q9966 LOCM 200-299MG/ML IODINE,1ML

Follow Up

tomorrow (Reason: discuss Pump implant)

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 10/24/2025 at 12:32 PM MDT

Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 10/23/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/24/2025

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

10/24/2025 **Claudia Hougland, CNP**

Chief Complaint

1. PM follow up after Baclofen Trial
2. I had really good relief with spasticity. It even helped my legs a ton which I didn't even know were a problem. I still had some issues in my arms but when I tried to make the spasticity worst, I wasn't able to. Overall I was happy with the relief

History of Present Illness

Disability indexing:
Disability scale PDI =22 Date of administration:10/24/25.

Pain pump:
Patient is s/p Pain pump trial The patient experienced over 50% pain relief without SE's. After discussion of the risks, benefits, and alternatives, they wish to move forward with the pain pump implant ASAP.

Yesterday during the trial, she felt happy with overall improvement in spasticity noting improvement in her legs which she had not previously recognized as problematic, she attempted to provoke spasms in her arms by holding a phone or remote but was unable to reproduce them.

PT evaluation of pre and post modified ashworth scale showed marked improvement post vs pre testing of BUE and the pain in her BLE was abolished as well.

Preop evaluation:
Anesthesia history No previous adverse events with anesthesia. No family history of adverse events with anesthesia. No history of difficult intubation . Functional Status Can easily climb a flight of stairs but would be somewhat limited due to pain, Denies previous MI/CHF, COPD, O2 use, Liver/kidney issues, neurological disorders/CVA, OSA - denies, Acid reflux/GERD - admits takes pantoprazole prn.

Injection:
Response to previous injection 9/10/25 bilateral C4/5 TFE - 20% relief.

Referral:
The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Current Medications

Taking

- Advil 200 MG Tablet 1 tablet with food or milk as needed Orally Three times a day
- Baclofen 10 MG Tablet 1 tablet as needed Orally TID , stop date 04/04/2026
- hydrOXYzine HCl 25 MG Tablet 1 tablet as needed Orally every 6 hours for severe itching
- Pregabalin 75 MG Capsule 1 capsule Orally bid , stop date 12/04/2025
- Zofran 4 MG Tablet 1 tablet Orally TID PRN NAUSEA

Tylenol 325 MG Tablet 1 tablet as needed Orally every 6 hrs
Medication List reviewed and reconciled with the patient

Medical History
- Healthy.

Surgical History/Major Diagnostic Procedure
- cervical fusion 2021
- hysterectomy 2023

Family History
- No Family History documented.

Social History
ETOH: socially
smoke: none.

Allergies
- Penicillin

Hospitalization
- No Hospitalization History.

Vital Signs
BP: **122/82**, HR: **74**, Ht: 65, Wt: **153**, BMI: **25.46**, Oxygen sat %: **94**, Ht-cm: 165.1 cm, Wt-kg: 69.4 kg.

Examination
Pain Exam:
General

Patient alert and oriented 3. Speech fluent. No acute distress noted. Posture upright but with mild increased tone noted in dependent limbs. Gait assisted.

Cranial Nerves:
Cranial nerves II-XII grossly intact. No facial asymmetry, dysarthria, or tongue deviation. Pupils equal, round, reactive to light. Extraocular movements intact.

Motor Examination:
Right upper extremity shows increased tone with mild flexor posturing at the elbow and wrist. Strength 4+/5 proximally and 4-/5 distally.
Left upper extremity tone normal, strength 5/5.
Both lower extremities show increased tone, greater on the right. Hip flexion 4/5, knee extension 4-/5, plantar flexion 4-/5 on right; left leg strength 4+/5 throughout.

Tone:
Spasticity present in the right upper extremity and both lower extremities.
Modified Ashworth Scale:

- Right upper extremity: Elbow 2, wrist 2, fingers 1+.
- Right lower extremity: Hip flexors 2, knee extensors 2, ankle plantarflexors 1+.
- Left lower extremity: Hip flexors 1+, knee extensors 2, ankle plantarflexors 1+.
  Catch noted on rapid passive movement, more pronounced with increased velocity. No clonus in upper extremities; 2-3 beat clonus at bilateral ankles.

Reflexes:
Biceps 3+ right, 2+ left.
Triceps 3+ right, 2+ left.
Brachioradialis 3+ right, 2+ left.
Patellar 4+ bilaterally.
Achilles 3+ bilaterally.
Neg clonus

General Examination:

HEART: regular rate and rhythm, no murmurs, rubs, gallops .
LUNGS: clear to auscultation bilaterally, good air movement .

Assessments

1. C5-C7 incomplete quadriplegia - G82.54 (Primary)
2. Dystonia - G24.9
3. Chronic pain - G89.29

Treatment

1. C5-C7 incomplete quadriplegia

LAB: PLATELET FUNCTION TEST (Ordered for 10/24/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
LAB: BMP (Basic Metabolic Panel) (Ordered for 10/25/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
LAB: CBC Auto Diff (Ordered for 10/25/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
LAB: MRSA BY NASAL SCREEN, NAT (SANFORD) (Ordered for 10/26/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
PROCEDURE: Pump Implantation (62362) (Ordered for 10/24/2025)
PROCEDURE: Pump catheter implant or revise (62350) (Ordered for 10/24/2025)

Notes: The patient had a successful pump trial with over 50% relief of the pain. They also experienced 50% improvement in function, better sleep, overall general increase in health. Neuropsych was passing. The patient would like to proceed with a pump implantation as soon as possible. We did discuss the risks, benefits, alternatives of the procedure -they include the same risks as the trial is the additional risk of anesthesia.

Preoperative labs today.

2. Dystonia

LAB: PLATELET FUNCTION TEST (Ordered for 10/24/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
LAB: BMP (Basic Metabolic Panel) (Ordered for 10/25/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
LAB: CBC Auto Diff (Ordered for 10/25/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
LAB: MRSA BY NASAL SCREEN, NAT (SANFORD) (Ordered for 10/26/2025) (Collection Date & Time - 10/24/2025 12:18 PM)
PROCEDURE: Pump Implantation (62362) (Ordered for 10/24/2025)
PROCEDURE: Pump catheter implant or revise (62350) (Ordered for 10/24/2025)

3. Chronic pain

Notes: Continue Lyrica, increase to 75 mg PO BID for nerve pain management.

I reviewed the PDMP reviewed today. The medications and intervals are consistent with expected results

4. Others

Start Pregabalin Capsule, 75 MG, 1 capsule, Orally, bid, 30 days, 60 Capsule, Refills 1
Continue Baclofen Tablet, 10 MG, 1 tablet as needed, Orally, TID, 90 days, 270 Tablet, Refills 1
Start Cephalexin Capsule, 250 MG, 1 capsule, Orally, TID, 7 days, 21 Capsule, Refills 0

Claudia Hougland, CNP
Claudia Hougland, CNP

ectronically signed by Claudia Hougland , CNP on 10/28/2025 at 10:40 AM MDT
Sign off status: Completed

RCMC Pain Medicine
101 East Minnesota Street
Rapid City, SD 577017758
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Claudia Hougland, CNP 10/24/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

RAPID CITY MEDICAL CENTER LLP

Pulliam, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

12/01/2025 **Tyler Ptacek MD**

Chief Complaint

1. Here for Epidural Blood Patch
2. Severe headache from dural puncture, failure of more than 72 hours of conservative treatments: caffeine, fluids, laying flat
3. I had really good relief with spasticity. It even helped my legs a ton which I didn't even know were a problem. I still had some issues in my arms but when I tried to make the spasticity worst, I wasn't able to. Overall I was happy with the relief

History of Present Illness

Disability indexing:

Disability scale PDI =22 Date of administration:10/24/25.

Injection:

The patient is here for a procedure for severe pain control after a dural puncture caused a severe positional headache. When the patient lays flat the headache resolves, when they are upright the pain is so unbearable that they are not able to accomplish even basic activities of daily living. They have attempted greater than 72 hours of conservative cares including caffeine, fluids, lying recumbent. Denies fevers, chills, nausea, vomiting

She is s/p TDD implant last Tuesday without complications besides this spinal leak. Response to previous injection 9/10/25 bilateral C4/5 TFE - 20% relief.

Referral:

The patient was kindly referred for evaluation of pain by Self referral. Vicky Pulliam presents with worsening pain and muscle spasms following C5-6-6-7 fusion due to car accidents in September 2019 and February 2020. The patient initially experienced relief post-surgery, but symptoms worsened within a year. The patient reports muscle spasms, twitches, and pain, particularly in her arm, mid-back, and neck. She has been taking 3200 mg of Advil and 4000 mg of Tylenol daily for two months, along with Lyrica, but the pain persists. Dr. Schubert prescribed Vicodin for severe pain. The patient notes a significant exacerbation of symptoms in the last two months, which she attributes to a recent bout of flu and possibly sleeping in an awkward position. She attempted to alleviate her symptoms with two massage sessions, but this resulted in worsening pain. The patient is considering more permanent pain management solutions, including a pain pump. She reports feeling tired and exhausted due to her ongoing symptoms. Her current condition is impacting her daily life, and she expresses concern about defining her future medical needs, particularly in the context of an upcoming trial related to her car accidents. Past surgical history significant for C5-6 6-7 fusion 2021 . Past medical history is significant for none reported . The pain started years ago . The pain is worsening . Rated as an NRS score of 8/10 at its worst , 3/10 at its best , 3/10 today . The pain is primarily in the neck, thoracic spine, right arm . It is described as aching, throbbing, dull, cramping, sharp, stabbing, constant . The pain is worse with laying down, lifting, . The pain is better with N/A - no activity modification makes the pain better . The pain is affecting the following ADL's Sleeping, working, household chores, making meals, dressing . Disability index scoring Pain disability index =28 (21-40 is moderate disability) . Conservative treatments Self-directed exercise 6+wks, Physical Therapy Gillette 2021-2022 6+wks, Chiropractic Gillette 2020-2025 6+wks . Medications tylenol, lyrica,ibuprofen, hydrocodone . Smoking status never smoker .

Vision Examination

Examination

General Examination:

HEART: regular rate and rhythm, no murmurs, rubs, gallops .
LUNGS: clear to auscultation bilaterally, good air movement .

Pain Exam:

General patient is miserable with severe pulsating headache with siting and standing that resolves with laying
CN 2-12 intact.

Assessments

1. Other reaction to spinal and lumbar puncture - G97.1 (Primary)
2. Other specified personal risk factors, not elsewhere classified - Z91.89
3. Pain, unspecified - R52

ocedures

Epidural blood patch using fluoroscopic guidance:

Pre/Post operative diagnosis Spinal headache, dural puncture headache.
Surgeon Tyler A. Ptacek.
Indications The patient has a severe and debilitating positional headache from a dural puncture and has failed 48 hours of conservative cares: fluids, caffeine, recumbency.
Informed Consent The risks, benefits, potential complications and alternatives were discussed with the patient as per the signed consent form, and informed consent was obtained. The patient has received information about the procedure and its risks. The physician personally confirmed the procedure, side, and site to be injected with the patient in the pre-procedure area. A time-out was taken prior to the start of the procedure to confirm the patient, the procedure and the site.
Procedure The patient was placed in the prone position on the fluoro table and an AP scout film was obtained to determine the exact level of entry. The level was chosen and landmarks were identified via fluoroscopy. A sterile prep and drape was undertaken on the back and over the antecubital fossa in the usual fashion. 1cc 1% Lidocaine was used to anesthetize the skin. A 3.5 inch x 20g Tuohy-type needle was advanced under intermittent fluoroscopic guidance towards the L3/4 interspace where loss of resistance to saline was noted. A lateral radiographic was obtained to confirm appropriate placement and 1cc of omnipaque 240 confirmed epidural spread. Meanwhile an assistant was drawing blood out of the antecubital fossa on the right using strict sterile technique. 30 cc of this blood was drawn and handed back to me. I did bolus this sterilely into the epidural space causing every 5 cc in order to prevent extreme pressure in the low back. The patient was able to tolerate the entire 30 cc bolus. Procedure was tolerated very well, there were no acute complications. He was taken to the recovery room where he was directed to lay reclined for 30 minutes prior to discharge.
Pre-procedure pain score 8.
Post-procedure pain score 0.

Procedure Codes

- 62273 TREAT EPIDURAL SPINE LESION
- A4550 SURGICAL TRAY
- 99195 PHLEBOTOMY
- Q9966 LOCM 200-299MG/ML IODINE,1ML
- 77003 FLUOROGUIDE FOR SPINE INJECT

Follow Up

3 Weeks (Reason: pain pump implant)

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 12/05/2025 at 12:30 PM MST

Sign off status: Completed

RCMC Pain Medicine
101 E MINNESOTA ST
RAPID CITY, SD 57701-7756
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 12/01/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

RAPID CITY MEDICAL CENTER LLP

Pulliam, Viktoriya
35 Y old Female, DOB: 12/06/1989
Account Number: 604899
147 OVERBROOK RD, GILLETTE, WY-82718-4098
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

12/04/2025 Tyler Ptacek MD

Chief Complaint
1. Here for pain pump refill and titration

History of Present Illness
Interval Hx:
Pain is: Stable. Post operative wound check Patient is here for a 10 day wound check s/p surgery, doing well, soreness is better. Denies F/C/N/V/D.
-
Excited to get the staples out.
Pain pump:
At the last visit: she had a post dural punture headache, we completed an EDBP with resultant resolution of the severe headache.

Vision Examination

Examination
Pain Exam:
Incision site(s) look clean, dry, intact. Minimal errythema, healing well.

Assessments
1. Postop check - Z09 (Primary)
2. Chronic pain syndrome - G89.4
3. Encounter for adjustment and management of infusion pump - Z45.1
4. Presence of intrathecal pump - Z97.8

Treatment
1. Postop check

Notes: I examined the patient's wound and removed the staples after cleansing the sites with betadyne and alcohol. I applied steristrips where needed. Patient was instructed to let these fall off on their own. Patient instructed to place a bandage daily as needed but that this was no longer needed. They will call back to the office if they have increasing pain, fever, chills, nausea, or any other symptoms

Procedures
Pain pump refill:
Procedure The patient was identified and the correct medication and dosage was verified. The pain pump was identified and was cleansed with Betadine and alcohol x3. The MEDTRONIC refill kit was opened. The refill needle was used to access the pain pump. This was attached to the extension tubing and the clamp was opened. After withdrawing the old medication, the clamp was locked and the old medication was discarded after witnessing the type of medication and quantity with the support staff. The new medication was identified with the patient. The medication was attached to the filter and all of the air bubbles were purged. It was then connected to the tubing and the clamp was released. Barbotage technique every 5 mL for a total of 20 cc placed into the pump. The pump was then interrogated and reprogramming was successful. A printout of the changes were given to the patient and scanned into the chart. The patient was then observed for a short while for signs or symptoms of a pocket fill or other adverse reactions. None were noted.
Pump MEDTRONIC SYNCHROMED.
Volume expected 19.9ml.
Volume obtained 19.9ml.
Medication taken out of the pump Water;, Medication Instillled into the pump Baclofen.
Concentration 400.0mcg/ml.
Reservoir volume 20ml.
Pump Flow mode SImple continuous.
Daily dose 50.0mcg/day.
Alarm Volume 2.0ml.

Refill date 01/23/26.
PATIENT ACTIVATOR PTM dose: 15mcg
24 hour max activations: 6
PTC Daily max: 139.39mcg
Lockout time (hours): 2.

Procedure Codes

- 62370 ANL SP INF PMP W/MDREPRG&FIL
- A4220 REFILL KIT IMPLANTABLE INFUS PUMP

Follow Up

prn. prn (Reason: worsening pain. for pump titration and/or refill)

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 12/05/2025 at 07:35 AM MST

Sign off status: Completed

RCMC Pain Medicine
101 E MINNESOTA ST
RAPID CITY, SD 57701-7756
Tel: 605-342-3280
Fax: 605-721-8439

Progress Note: Tyler Ptacek MD 12/04/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)
Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

RAPID CITY MEDICAL CENTER, LLP

Pulliam, Viktoriya
36 Y old Female, DOB: 12/06/1989
Account Number: 604899
100 W HOGEYE DR, GILLETTE, WY-82716-4822
Home: 503-208-1326
Guarantor: Pulliam, Viktoriya Insurance: AETNA
Appointment Facility: RCMC Pain Medicine

01/05/2026 **Tyler Ptacek MD**

Chief Complaint
1. PM Pump Titration

History of Present Illness
Pain pump:
At the last visit: the patient has continued with the IT pump for spasticity.
Follow up:
Since the last visit Vicky Pulliam is a patient with spinal cord injury who returns for follow-up of her intrathecal baclofen pump management. She reports that things have generally settled down since her surgery, though she notes numbness below the surgical scar.

Regarding her intrathecal baclofen therapy, she describes mixed effectiveness. On December 30th or 31st, she experienced severe symptoms reminiscent of when her condition first started, with significant spasticity and spasms. The spasms in her neck feel sharp and pulling, like "it's pulling something" or "that bone spur or something." She was concerned the pump had disconnected and administered multiple boluses back-to-back until symptoms improved. She believes her current baseline dose is insufficient.

She reports a notable side effect of chest heaviness lasting about an hour after bolus administration, which reassures her the pump is functioning. The effectiveness of boluses varies - sometimes they provide just enough relief, other times they work well. She averages 4-5 boluses daily, though during the severe episode she used the maximum of 6. She finds staying ahead of symptoms with regular boluses throughout the day more effective than waiting until symptoms worsen.

Sleep has been problematic again, as she cannot sleep on one side due to a pinched sensation and pulling pain. Recent life stressors include a chaotic move where their moving company cancelled, leaving her husband to handle everything while she was limited to 10-pound weight restrictions.

She successfully discontinued Lyrica and remains off oral baclofen. During the severe episode on December 31st and January 1st, she temporarily resumed Lyrica with minimal benefit the first day and slight improvement the second day, but has since stopped it again. She prefers to avoid medications that cause constipation and mental fogginess to maintain clear-headedness..

Examination
Pain Exam:
General Mild to moderate distress due to pain, Respiratory drive normal, Alert and oriented x 3, No gross motor deficits, Slow ambulation due to pain.

Assessments
1. Encounter for adjustment and management of infusion pump - Z45.1 (Primary)
2. Presence of intrathecal pump - Z97.8
3. Spasticity - R25.2

Treatment
1. Encounter for adjustment and management of infusion pump

Notes: Patient reports variable effectiveness of current intrathecal baclofen dosing with episodes of severe spasticity and spasms reminiscent of initial presentation, particularly on December 30th-31st. Currently experiencing neck spasms described as sharp pulling sensations concerning for bone spur regrowth. Patient reports chest heaviness lasting approximately one hour after bolus administration, which is consistent with medication distribution along the spinal cord. Currently averaging 4-5 boluses daily, with ability to tolerate up to 6 boluses without significant sedation. Patient desires optimization of baseline dosing to reduce reliance on bolus dosing while maintaining symptom control. Recent stressors including moving may be contributing to increased spasticity.
Plan:
- Increase baseline intrathecal baclofen to 140 micrograms daily
- Continue current bolus dosing regimen with 6 boluses available as needed

- Patient counseled that chest tightness is expected side effect from bolus administration and should resolve with higher baseline dosing
- Target dose range for spinal cord injury patients is 200-600 micrograms daily

2. Others

Notes: Patient successfully discontinued Lyrica with brief resumption on December 31st and January 1st during severe spasticity episode. Patient reports Lyrica causes constipation and cognitive impairment, with minimal benefit for symptom control. Patient prefers to remain off oral medications to maintain clear cognition.
Plan:
- Continue off Lyrica
- Avoid oral medications when possible Patient reports numbness below surgical scar which appears stable since surgery. No acute concerns regarding surgical site healing or complications.
Plan:
- Continue monitoring

Procedures

Pain pump interrogation and reprogramming:

Procedure The pain pump was interrogated. After medical decision-making, the pain pump was changed appropriately. Printout was given to the patient as well as put into the chart.
Medication in the pump Baclofen.
Concentration 400.0mcg/ml.
Previous settings 50.00mcg/day.
Updated pump settings 140.06mcg/day.
Reservoir volume 12.4ml.
Alarm volume 2.0ml.
Refill date 1/23/26
PTM:
Bolus dose: 15.00mcg
Max Bolus dose: 139.39mcg
Bolus per day: 6/day
Lockout 2 hour.

Procedure Codes

- 62368 ANALYZE SPINE INFUSION PUMP

Tyler A. Ptacek, MD

Electronically signed by Tyler Ptacek , MD on 01/06/2026 at 09:13 AM MST

Sign off status: Completed

RCMC Pain Medicine
101 E MINNESOTA ST
RAPID CITY, SD 57701-7756
Tel: 605-342-3280
Fax: 605-721-8439

PULLIAM, Viktoriya **DOB:** 12/06/1989 (36 yo F) **Acc No.** 604899 **DOS:** 01/05/2026

Progress Note: Tyler Ptacek MD 01/05/2026

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

Generated for Printing/Faxing/eTransmitting on: 01/12/2026 03:41 PM MST

Plastic Surgical Center of Rapid City, Prof., LLC | 3615 5th St, Ste 100 | Rapid City, SD 57701 | Phone: 605-343-6714 | Fax: 605-343-2130

Plastic Surgical Center RAPID CITY

Name: Pulliam, Viktoriya
Acct #: 12200
DOB: 12/06/1989 (35yr)

DOS: 11/25/2025
Address: 1

INTRATHECAL DRUG DELIVERY PUMP AND CATHETER IMPLANTATION WITH MEDTRONIC SYNCHROMED III

PRE OPERATIVE DIAGNOSIS: Incomplete C5-7 Quadriplegia with severe spasticty/dystonia, Chronic pain syndrome

POST OPERATIVE DIAGNOSIS: Same

SURGEON: Tyler A. Ptacek MD

ANESTHESIA: GETA with LMA

EBL: Minimal

Catheter Tip Location: T1

Indication for the procedure: The patient has a history of severe, intractable, and debilitating pain for over 12 months. There has been a failure of extensive conservative cares over 6 months including physician directed HEP, PT, Medications, activity modifications, Injections, and surgical consultations. The patient has no exclusion criteria for proceeding with the implantation as documented in the neuropsychological evaluation. There are no surgical lesions present. A pump trial yielded over 75% improvement in spasticity/dystonia without significant SE's. She also had greatly improved modified ashworth scales with her PT pre and post IT baclofen trial.

Informed Consent: The diagnosis and possible alternative diagnosis, the nature of the proposed treatment/procedure, a description of the treatment/procedure, risks associated with the treatment/procedure, possible benefits of the treatment/procedure, possible alternative methods of care and associated risks, and the risk of no treatment were thoroughly discussed with the patient. An appropriate consent from was signed, indicating the patient understands the procedure and its possible complications, risks, and alternatives

OPERATIVE COURSE:

The patient was brought to the operative room. Anesthesia was initiated and they were placed in the left lateral decubitus/supine position. Pressure areas were carefully padded at the head/neck, arms, legs, and an axillary roll was placed. The skin was marked and then carefully prepped with alcohol and Duraprep. The skin was draped in the appropriate manner and covered with ioban dressing. The surgical incision sites were identified. Using the AP view with fluoroscopy, the L3 interspinous level was identified. Surgical incision was made in the lumbar area first on the skin using the #10 surgical blade. Hemostasis was achieved using electrocautery. Combining electrocautery and blunt dissection, dissection was continued until the upper fascia was identified. The spinous processes at the L2 level were identified using fluoroscopy. The skin was kept in position using a retractor. Utilizing the 15g tuohy needle a spinal tap was obtained at the L2/3 level with careful lateral fluoroscopic view confirming that only the posterior area of the intrathecal space was entered. Once the intrathecal space was entered, the intrathecal catheter was inserted through the needle and advanced in the intrathecal space to approximately at T4/5 posteriorly. After the stylet was withdrawn there was free cerebrospinal fluid flow through the catheter. Next a "Figure of Eight" suture technique was performed at the needle insertion site encompassing the fascia using an Ethibond suture. Following this, the needle was withdrawn and the cerebrospinal fluid flow continued to be adequate. The intrathecal catheter was previously secured at the interspinous ligament using two 2-0 Ethibond sutures and the anchoring device. The catheter was then lightly clamped and a second incision was made in the right abdominal area. In a similar fashion, an approximate seven cm incision was made using a #10 blade. Hemostasis was achieved using electrocautery. Blunt dissection for the IT pump was made approximately one cm deep to the skin surface. Utilizing the ventriculoperitoneal shunt tunneling device, a tunnel was made between the first and second incision and the

intrathecal catheter was placed through the tunnel and exited through the pocket incision. The tip of the catheter was attached to the 20cc SYNCHROMED 3 pump. Next with aspiration of the side port using the appropriate needle, there was adequate CSF flow. I also pressure tested the system which showed no leaks and an intact intrathecal drug delivery device catheter. The redundant catheter was coiled under the pump and the pump was then placed in the pocket. Irrigation was employed at both incisions using antibiotic and saline. The skin was closed in two layers using 2-0 Vicryl interrupted sutures for the subcutaneous tissues. Next staples were applied to the epidermal layer, triple antibiotic ointment and a non-occlusive dressing was placed over the staples. The pump was programmed to run sterile water which was already present in the pump. The patient tolerated the procedure well and was transferred to the recovery room in stable condition. They were given postoperative antibiotics and will follow up in the clinic in 10 days. Postoperative instructions were given to the patient's significant other/caregiver

Electronically Signed By: Tyler Ptacek, MD
11/25/2025 11:05

RAPID CITY MEDICAL CENTER, LLP

PULLIAM, Vicky
35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Proctor, Ashley A

Date: 07/22/2025
Time: 11:06 AM

Reason PM butrans

Message patient wanted to make sure both meds were loaded/sent for her that were discussed during her appt. loaded butrans, please verify dosage. thanks!

Action Taken Ptacek, Tyler A 07/23/2025 07:54:01 AM MDT > we discussed trialing baclofen first, I asked her to research butrans as we gather more records and plan for scs vs tdd. She has a ct myelo scheduled soon
Proctor, Ashley A 07/23/2025 08:27:08 AM MDT > notified pt

Refills Start Butrans Patch Weekly, 5 MCG/HR, Transdermal, 1 patch to skin

Provider: Ptacek, Tyler A 07/22/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Vicky **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 07/28/2025

RAPID CITY MEDICAL CENTER, LLP

PULLIAM, Vicky
35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Bartling, Lorena D

Date: 07/28/2025
Time: 04:05 PM

Caller patient

Reason PM baclofen reaction

Message patient says her tongue feels like it is burned she is congested, her throat is burning like she swallowed water wrong and is wondering if it is from the baclofen or another medication she is taking please call 503-208-1326

Action Taken
Hanson, Kayla L 07/29/2025 08:33:31 AM MDT > ATC, unable to leave VM due to mailbox full.
Hanson, Kayla L 07/30/2025 08:18:07 AM MDT > ATC, unable to leave VM due to mailbox full.
Hanson, Kayla L 07/31/2025 08:46:39 AM MDT > ATC, unable to reach pt. Will await for return phone call.

Provider: Ptacek, Tyler A 07/28/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Vicky **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 08/01/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Vicky
35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Weisz, Nechola M

Date: 08/01/2025
Time: 12:04 PM

Caller Patient

Reason PM refill

Message Patient is calling to request a refill for Baclofen 10 MG Tablet. Please send to Walgreens in WY

Action Taken Hanson, Kayla L 08/01/2025 12:12:37 PM MDT > Script sent.

Refills Refill Baclofen Tablet, 10 MG, Orally, 90 Tablet, 1 tablet as needed, TID, 30 days, Refills=1

Provider: Ptacek, Tyler A 08/01/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 08/08/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Bartling, Lorena D

Date: 08/08/2025
Time: 01:54 PM

Caller patient

Reason PM call back

Message patient called and would like a call back about some things that are going on please call her back at 503-208-1326

Action Taken Hanson, Kayla L 08/11/2025 09:37:09 AM MDT > Vikcy is wondering if you think the baclofen is causing her side effects. Pharmacist tells her it's a 1% chance. She is having a lot of congestion, burning in her throat/mouth. She states she thought about it and has had this prior to starting baclofen, but it is definitely exacerbated. She states laying down helps her congestion. Says the baclofen is causing her to be loopy and forgetful as well. The baclofen is helping her daily function and she has taken much less tylenol and ibuprofen. She is wanting your opinion on if she should continue or if you would want her to trial something else? She has follow-up on 8/18.
Ptacek, Tyler A 08/11/2025 09:56:27 AM MDT > yes go to 1/2 doses on the baclofen for 1 week and measure responses, I think a baclofen trial and the surgical 3rd opinion would be helpful, pump may be the best option but I would like her to see a surgeon at BHO for the OPLL
Hanson, Kayla L 08/11/2025 10:43:48 AM MDT > pt. notified

Provider: Ptacek, Tyler A 08/08/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 08/22/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Koscak, Matejah L

Date: 08/22/2025
Time: 10:05 AM

Caller patient

Reason PM procedure questions

Message Patient calling states that when she had seen Dr Woodruff on Wednesday he had suggested she get an epidural done with Ptacek before proceeding with anything else. Is this something we can get in for patient or will patient need to be seen in office prior? States a referral should have been sent as well from Woodruff but nothing shows in chart for this. Can we also send CT scan results to Black Hills Ortho for Dr Woodruff?
Please call patient back at 503-208-1326 for further discussion.

Action Taken Slone, Amanda M 08/22/2025 10:35:14 AM MDT > From Dr. Woodruff note - "My recommendation at this point is bilateral C4-5 transforaminal epidural steroid injections with Dr. Ptacek. She can call me 7 to 10 days later to let me know how it helps. If she gets significant relief with these injections but they are short-lived then we could possibly consider a fusion at the C4-5 level. If she gets no relief then I think we would have to assume that a lot of her symptoms are due to her spinal cord myelomalacia and problems related to her initial injury. At that point, I think that the pain pump could be an option."
Ptacek, Tyler A 08/22/2025 02:55:39 PM MDT > Ok please order bilateral C4/5 TFE, please have them push the CT scans to BHO
Bertolotto, Casey L 08/22/2025 03:33:21 PM MDT > The Future Procedure order Cervical TFE (64479) BT CLR RQD with order date 08/22/2025 has been placed.
Bertolotto, Casey L 08/22/2025 03:33:26 PM MDT > please have them push the CT scans to BHO
Bertolotto, Casey L 08/26/2025 12:00:28 PM MDT > Called patient and advised that she would need to call where she got the CT and have them send for her. I talked to her about the options and she states that she would like to make appt to go over next steps. Scheduled

Chief Complaint
1. PM procedure questions

Vital Signs

Physical Examination

Assessments

1. Cervical radiculopathy - M54.12 (Primary)

Treatment

1. Cervical radiculopathy
PROCEDURE: Cervical TFE (64479) BT CLR RQD (Ordered for 08/22/2025)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 08/22/2025

Provider: Ptacek, Tyler A 08/22/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 09/23/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya

35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Hunt, Janessa

Telephone Encounter

Answered by Koscak, Matejah L

Date: 09/23/2025
Time: 09:32 AM

Caller patient

Reason PM medication refill

Message Patient is requesting a refill of Lyrica 50 mg to Walgreens in Gillette WY.

Patient advised of 48 hour processing time and to contact pharmacy for status.

Action Taken
Bertolotto, Casey L 09/23/2025 09:56:10 AM MDT > I do not see that we have filled her Lyrica. Are you ok with filling??
Hunt, Janessa 09/23/2025 10:48:57 AM MDT > she has been having filled by provider in WY. We can prescribe if she would like us to take over
Bertolotto, Casey L 09/23/2025 10:58:43 AM MDT > Loaded
Hunt, Janessa 09/23/2025 11:59:07 AM MDT > rx sent

Refills Refill Lyrica Capsule, 50 MG, Orally, 30, 1 capsule, daily at night, 30 days

Provider: Hunt, Janessa 09/23/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/21/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya

35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Koscak, Matejah L

Date: 10/21/2025
Time: 11:13 AM

Caller patient

Reason PM questions about upcoming trial

Message Patient calling with questions about her upcoming pump trial she would like to discuss with nurse about, please call back at 503-208-1326 whenever possible.

Action Taken Bertolotto, Casey L 10/21/2025 12:35:02 PM MDT > Called patient and clarified procedure with her for Thursday. All questions answered and no further needs at this time. Thank

Provider: Ptacek, Tyler A 10/21/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/09/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya
35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Bertolotto, Casey L

Date: 10/09/2025
Time: 02:01 PM

Reason PM: MRI

Action Taken

Bertolotto, Casey L 10/09/2025 02:01:56 PM MDT > Do we need to have her Lumbar MRI done before she does her pump Trial??
Ptacek, Tyler A 10/10/2025 07:09:59 AM MDT > yes ordered in last note
Bertolotto, Casey L 10/10/2025 08:42:03 AM MDT > ATC patient to advise we do need MRI before the Trial. Patient is scheduled for 23rd for her Trial.

Norkia could you Check on auth for this MRI ASAP
Fletcher, Norika K 10/10/2025 09:41:57 AM MDT > Initiated request, but it's needing to go to an eligility review due to the facility, because it wasn't in their system and they had to add it. This can take up to 24hrs. I wasn't able to find any other facilities in WY that were coming up as in-network (only Sheridan VA). Checked Rapid as well and Im only finding MHRC. I can update the facility, or can wait for the review to come back; it doesn' typically take that long, just depends. It may or may not also need additional time for a medical necessity review, IF it doesn't auto-authorize. The facility check has to be done first before it moves onto that. Let me know if there are any questions on this.
Weisz, Nechola M 10/10/2025 09:47:12 AM MDT > Patient returning call to Casey
Roberts, Nicole Y 10/10/2025 09:50:31 AM MDT > Patient returning call to Casey
Bertolotto, Casey L 10/10/2025 10:04:24 AM MDT > Called patient and advised that we are working on Auth for MRI and will be in touch as soon as auth is approved. Will keep this open for watch for auth. If we have to will reschedule Trial. Thanks
Bertolotto, Casey L 10/14/2025 08:41:11 AM MDT > Still pending
Hanson, Kayla L 10/15/2025 07:28:09 AM MDT > MRI faxed this morning to Campbell County - also received psych eval
Bertolotto, Casey L 10/15/2025 08:36:52 AM MDT > ATC, VM was full. Thanks
Koscak, Matejah L 10/15/2025 11:26:13 AM MDT > patient returning call
Bertolotto, Casey L 10/15/2025 11:55:29 AM MDT > Called patient and advised. I will keep this open to keep eye on MRI results and also reminder to order Baclofen. I will call patient tomorrow to see if she got the MRI scheduled.
Bertolotto, Casey L 10/17/2025 08:37:48 AM MDT > Called to find out if she scheduled MRI. No option to leave VM.
Koscak, Matejah L 10/17/2025 08:39:56 AM MDT > Patient called back, stating she had gotten it scheduled and done yesterday 10/16, says we should be receiving results soon today
Hanson, Kayla L 10/17/2025 10:30:47 AM MDT > Noted, will await results. If we don't receive by Monday will need to call.
Hanson, Kayla L 10/17/2025 12:54:42 PM MDT > MRI in chart. OK to proceed with baclofen trial?
Ptacek, Tyler A 10/17/2025 01:35:14 PM MDT > yes
Hanson, Kayla L 10/17/2025 01:41:08 PM MDT > FYI- we are good to order baclofen
Bertolotto, Casey L 10/20/2025 08:55:35 AM MDT > Baclofen ordered.

Provider: Ptacek, Tyler A 10/09/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya

35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Bertolotto, Casey L

Date: 10/09/2025
Time: 01:57 PM

Reason PM: baclofen Trial

Message baclofen 50mcg

Bertolotto, Casey L 10/09/2025 01:53:45 PM MDT > Scheduled for 8am and noon with Hansen PT. Called patient and scheduled and advised of plan and address for Hansen.

Action Taken Bertolotto, Casey L 10/09/2025 01:58:57 PM MDT > Faxed referral to Hansen

Chief Complaint

1. PM: baclofen Trial

Vital Signs

Physical Examination

Treatment

1. Others

Referral To:
Reason:Patient is scheduled - - - - for a baclofen trial. Please eval before and after with modified ashworth scale.

Provider: Ptacek, Tyler A 10/09/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

PULLIAM, Viktoriya **DOB:** 12/06/1989 (35 yo F) **Acc No.** 604899 **DOS:** 10/06/2025

RAPID CITY MEDICAL CENTER LLP

PULLIAM, Viktoriya

35 Y old Female, DOB: 12/06/1989
147 OVERBROOK RD, GILLETTE, WY, US 82718-4098
Home: 503-208-1326
Provider: Ptacek, Tyler A

Telephone Encounter

Answered by Bowman, Karolyn M

Date: 10/06/2025
Time: 11:08 AM

Caller patient

Reason MRI

Message At check out today, Victoriya requesting that MRI be ordered with Campbell County Hospital, Gillette WY.

Action Taken Hanson, Kayla L 10/06/2025 11:11:57 AM MDT > Updated order.

Provider: Ptacek, Tyler A 10/06/2025

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

CAMPBELL COUNTY HEALTH

Outpatient Imaging
508 Stocktrail Ave.
Suite C, North Entrance
Gillette, WY 82716
Phone #: 307.688.1600
Fax #: 307.688.1640

MRI Report

Patient: PULLIAM,VIKTORIYA A
MR #: M000325769
URN #: E00260914
Acct:V00110103603 Loc: WORIRAD
DOB: 12/06/1989 Age/Sex: 35 / F
Date of Service: 06/17/25
Attending Prov: Sara M Thronburg FNP
Ordering Prov: Sara M FNP Thronburg
Acc. #(s): A0000598307
CPT: 72141

cc: Doctor,No
Thronburg,Sara M FNP~

EXAM: Cervical spine MRI without contrast

REASON FOR EXAM: CERVICAL DISC DEGENERATION

HISTORY: Neck and right arm numbness. History of fusion in 2020.

COMPARISON: Brachial plexus MRI 6/2/2025

PROCEDURE: Multiplanar multisequence MRI images of the cervical spine were obtained including sagittal T1, T2 and STIR sequences, axial 3D TSE and oblique T2 TFE sequences from C3 through T1 at 3-4 mm intervals without contrast. Field strength 1.5 Tesla.

FINDINGS:

Craniocervical and atlantodental alignment are preserved. Straightening of cervical lordosis. No spondylolisthesis. Vertebral body heights are maintained. Status post ACDF from C5-C7. No evidence of hardware complication. No bone marrow edema or suspicious osseous lesions.

The imaged posterior fossa structures are unremarkable.

Small focus of high T2 signal within the left ventral cord at the inferior C5 level (series 9 image 29).. Just inferior to this, at the C5-C6 level, there are punctate foci of T2 hyperintense signal within the left and right central cord (series 9 image 32). No evidence of cord edema. No additional signal abnormalities within the cervical cord.

No potentially concerning extraspinal findings.

Level by level findings:

C2 - C3: No canal or foraminal stenosis.

C3 - C4: No canal stenosis. Bilateral uncovertebral and facet arthropathy, causing mild-moderate right and mild left foraminal narrowing.

C4 - C5: Posterior disc-osteophyte complex, causing mild canal stenosis. Bilateral uncovertebral and facet arthropathy, causing moderate right and mild left foraminal stenosis.

C5 - C6: At the C6 level, there is a central signal abnormality along the ventral thecal sac, causing moderate canal stenosis and deforming the ventral cord. No foraminal stenosis.

C6 - C7: No canal or foraminal stenosis.

Additional copy
Imaging 0617-0079

Campbell County Health Outpatient Imaging
Patient name: PULLIAM,VIKTORIYA A
DOB: 12/06/1989
Account #: V00110103603

C7 - T1: No canal or foraminal stenosis. Mild bilateral facet arthropathy and mild disc bulge.

ORDER #: 0617-0013 MR/MRI cervical spine wo con
IMPRESSION:
1. Small foci of high T2 signal within the left ventral cord at the C5 level and within the left and right central cord at the C5-C6 level, favored to reflect myelomalacia.
2. Moderate canal stenosis at the C6 level, possibly from OPLL.
3. Moderate right C5 foraminal stenosis.
4. Additional findings as described.
-Electronically Signed By: Tyler Baldwin
-Electronically Signed On: 6/17/2025 2:41 PM

Some sections of this report may have been generated using a voice to text program. Every effort is made to correct errors, if mistakes are found they should be taken in context.

Dictated/Signed By: <Electronically signed by Tyler J Baldwin MD>06/17/25 1441

DD/DT: 06/17/25 1425
TD/TT: 06/17/25 1425 Transcriptionist: BTJ

Additional copy
Imaging 0617-0079

RCMC South Lab
101 East Minnesota Street Rapid City, SD 577017758
l: 605-342-3280 Fax: 605-721-8439

Claudia Hougland, CNP
Pain Medicine

tient Information: **01/12/2026**

Name: Pulliam, Viktoriya
DOB: 12/06/1989, 36Y Sex: Female
Address: 1
Home Phone:
Cell Phone: 5

Order Information:
Order Date: 10/24/2025
Assessments: C5-C7 incomplete quadriplegia - G82.54, Dystonia - G24.9
Lab: **PLATELET FUNCTION TEST**
Fasting: No
Specimen: Collection Date: 10/24/2025 Time:12:18 PM Source:Blood&Blood
Clinical Info:

Name	Value	Reference Range
Platelet Function EPI	126	20-193 sec
PLT COUNT/PLT FUNCT TEST	315	140-350 10*3/uL
Platelet Function ADP		

Result:
Received Date: 10/24/2025
Notes:

Patient Name: Pulliam, Viktoriya, 36Y, F, DOB: 12/06/1989

Pulliam, Viktoriya, 35 Y, F, 12/06/1989

RCMC South Lab
101 East Minnesota Street, Rapid City, SD 577017758
T: 605-342-3280

FINAL RESULT

Accession ID: 15205248 Specimen Source: Blood&Blood	Ref ID: 15205248 Specimen Description: Blood&Blood
Order Date: 10/24/2025 Collection Date: 10/24/2025 12:18:00	Received: 10/24/2025 14:54:01 Report: 10/24/2025 14:50:56
Requesting Physician: Hougland, Claudia	Ordering Physician: Hougland, Claudia

PLATELET FUNCTION TEST

	NAME	VALUE	REFERENCE RANGE
F	Platelets	315	140-350 (10*3/uL)
F	Platelet Function EPI	126	20-193 (sec)

Results indicate normal platelet function.

PERFORMING LAB: MONUMENT HEALTH LABORATORY - RAPID CITY HOSPITAL, MONUMENT HEALTH LABORATORY - RAPID CITY HOSPITAL, 353 FAIRMONT BLVD., RAPID CITY SD

Pulliam, Viktoriya, 35 Y, F, 12/06/1989 Accession ID: 15205248

RCMC South Lab
101 East Minnesota Street Rapid City, SD 577017758
l: 605-342-3280 Fax: 605-721-8439

Claudia Hougland, CNP
Pain Medicine

tient Information: **01/12/2026**

Name: Pulliam, Viktoriya
DOB: 12/06/1989, 36Y, Sex: Female
Address:
Home Phone: [illegible]
Cell Phone: 5[illegible]

Order Information:
Order Date: 10/24/2025
Assessments: C5-C7 incomplete quadriplegia - G82.54, Dystonia - G24.9
Lab: **MRSA BY NASAL SCREEN, NAT (SANFORD)**
Fasting: No
Specimen: Collection Date: 10/24/2025 Time:12:18 PM Source:257261003
Clinical Info:

Name	Value	Reference Range
MRSA BY NAD, NASAL	Not Detected	Not Detected

Result:
Received Date: 10/26/2025
Notes:

Patient Name: Pulliam, Viktoriya, 36Y, F, DOB: 12/06/1989

Pulliam, Viktoriya, 35 Y, F, 12/06/1989

RCMC South Lab
101 East Minnesota Street, Rapid City, SD 577017758
T: 605-342-3280

FINAL RESULT

Accession ID: 1353931 Specimen Source: 257261003	Ref ID: 4390O15205248 Specimen Description: 257261003
Order Date: 10/24/2025 Collection Date: 10/24/2025 12:18:00	Received: 10/26/2025 11:16:04 Report: 10/26/2025 11:15:00
Requesting Physician: Hougland, Claudia	Ordering Physician: Hougland, Claudia

MRSA BY NASAL SCREEN, NAT (SANFORD)

	NAME	VALUE	REFERENCE RANGE
F	MRSA BY NAD, NASAL	Not Detected	Not Detected

MRSA DNA not detected. MRSA nasal colonization unlikely.

This test was performed by polymerase chain reaction (PCR) on the GeneXpert instrument.

Testing Performed At: SANFORD REFERENCE LABORATORY SIOUX FALLS 2301 E.60TH ST. NORTH SIOUX FALLS, SD 57104

Pulliam, Viktoriya, 35 Y, F, 12/06/1989 Accession ID: 1353931

RCMC South Lab
101 East Minnesota Street Rapid City, SD 577017758
: 605-342-3280 Fax: 605-721-8439

Claudia Hougland, CNP
Pain Medicine

01/12/2026

Patient Information:

Name:	Pulliam, Viktoriya
DOB:	12/06/1989, 36Y, Sex: Female
Address:	1
Home Phone:	
Cell Phone:	

Order Information:

Order Date:	10/24/2025
Assessments:	C5-C7 incomplete quadriplegia - G82.54, Dystonia - G24.9
Lab:	**CBC Auto Diff**
Fasting:	No
Specimen:	Collection Date: 10/24/2025 Time:12:18 PM Source:WB
Clinical Info:	

Name	Value	Reference Range
White Blood Count	8.5	4.5-10.5 K/uL
Red Blood Count	4.76	4.20-5.40 M/uL
Hemoglobin	14.1	12.6-16.0 g/dL
Hematocrit	43.0	37.0-47.0 %
Mean Corpuscular Hemoglobin	30	27-34 pg
Mean Corpuscular Hemoglobin Concentration	33	31-37 g/dL
Mean Corpuscular Volume	90	82-98 fL
Red Cell Distribution Width	**11.9 L**	**12.0-14.0 %**
Platelet	325	150-450 K/uL
Neutrophil %	51.5	42.0-75.0 %
Lymphocyte %	37.9	21.0-51.0 %
Monocyte %	7.2	4.0-15.0 %
Eosinophil %	2.8	0.0-5.0 %
Basophil %	0.4	0.0-2.0 %
Immature Gran%	0.2	0.0-1.0 %
Absolute Neutrophil Count	4.35	1.50-7.00 K/uL
Absolute Lymphocyte Count	3.20	0.70-5.00 K/uL
Absolute Monocyte Count	0.61	0.00-0.90 K/uL
Absolute Eosinophil Count	0.24	0.00-0.40 K/uL
Absolute Basophil Count	0.03	0.00-0.10 K/uL
Absolute Immature Gran Count	0.02	<0.10 K/uL

Result:
Received Date: 10/24/2025
Notes:

Patient Name: Pulliam, Viktoriya, 36Y, F, DOB: 12/06/1989

Pulliam, Viktoriya, 35 Y, F, 12/06/1989

RCMC South Lab
101 East Minnesota Street, Rapid City, SD 577017758
T: 605-342-3280

FINAL RESULT

Accession ID: 1353930 Specimen Source: WB	Ref ID: 4390O15205248 Specimen Description: WB
Order Date: 10/24/2025 Collection Date: 10/24/2025 12:18:00	Received: 10/24/2025 12:32:05 Report: 10/24/2025 12:31:00
Requesting Physician: Hougland, Claudia	Ordering Physician: Hougland, Claudia

CBC Auto Diff

	NAME	VALUE	REFERENCE RANGE
F	WBC	8.5	4.5-10.5 (K/uL)
F	RBC	4.76	4.20-5.40 (M/uL)
F	HGB	14.1	12.6-16.0 (g/dL)
F	HCT	43.0	37.0-47.0 (%)
F	MCV	90	82-98 (fL)
F	MCH	30	27-34 (pg)
F	MCHC	33	31-37 (g/dL)
F	PLT	325	150-450 (K/uL)
F	**RDW**	**11.9 L**	**12.0-14.0 (%)**
F	NEUT%	51.5	42.0-75.0 (%)
F	LYMPH%	37.9	21.0-51.0 (%)
F	MONO%	7.2	4.0-15.0 (%)
F	EOS%	2.8	0.0-5.0 (%)
F	BASO%	0.4	0.0-2.0 (%)
F	IMMATURE GRAN%	0.2	0.0-1.0 (%)
F	NEUT#	4.35	1.50-7.00 (K/uL)
F	LYMPH#	3.20	0.70-5.00 (K/uL)
F	MONO#	0.61	0.00-0.90 (K/uL)
F	EOS#	0.24	0.00-0.40 (K/uL)
F	BASO#	0.03	0.00-0.10 (K/uL)
F	IG#	0.02	<0.10 (K/uL)

Test performed at:
RCMC South Lab
101 East Minnesota St.
Rapid City, SD 57701 (605)348-2273

RCMC South Lab
101 East Minnesota Street Rapid City, SD 577017758
l: 605-342-3280 Fax: 605-721-8439

Claudia Hougland, CNP
Pain Medicine

tient Information: **01/12/2026**

Name: Pulliam, Viktoriya
DOB: 12/06/1989. 36Y, Sex: Female
Address: [illegible] 82716-4822
Home Phone:
Cell Phone: 5[illegible]

Order Information:
Order Date: 10/24/2025
Assessments: C5-C7 incomplete quadriplegia - G82.54, Dystonia - G24.9
Lab: **BMP (Basic Metabolic Panel)**
Fasting: No
Specimen: Collection Date: 10/24/2025 Time:12:18 PM Source:SER
Clinical Info:

Name	Value	Reference Range
Calcium	9.3	8.7-10.6 mg/dL
Glucose	89	70-100 mg/dL
Blood Urea Nitrogen	10	7-25 mg/dL
Creatinine	0.7	0.6-1.3 mg/dL
Sodium	140	134-145 mmol/L
Potassium	4.0	3.5-5.0 mmol/L
Chloride	106	98-111 mmol/L
Carbon Dioxide	29	21-31 mmol/L
GFR	93.21	

sult:
ceived Date: 10/24/2025
Notes:

Patient Name: Pulliam, Viktoriya, 36Y, F, DOB: 12/06/1989

Pulliam, Viktoriya, 35 Y, F, 12/06/1989

RCMC South Lab
101 East Minnesota Street, Rapid City, SD 577017758
T: 605-342-3280

FINAL RESULT

Accession ID: 1353929 Specimen Source: SER	Ref ID: 4390O15205248 Specimen Description: SER
Order Date: 10/24/2025 Collection Date: 10/24/2025 12:18:00	Received: 10/24/2025 16:10:08 Report: 10/24/2025 16:09:00
Requesting Physician: Hougland, Claudia	Ordering Physician: Hougland, Claudia

BMP (Basic Metabolic Panel)

	NAME	VALUE	REFERENCE RANGE
F	CA	9.3	8.7-10.6 (mg/dL)
F	GLU	89	70-100 (mg/dL)
F	BUN	10	7-25 (mg/dL)
F	CREA	0.7	0.6-1.3 (mg/dL)
F	NA+	140	134-145 (mmol/L)
F	K+	4.0	3.5-5.0 (mmol/L)
F	Cl-	106	98-111 (mmol/L)
F	CO2	29	21-31 (mmol/L)
F	GFR	93.21	

Estimated GFR calculated using the IDMS-traceable MDRD study equation with the result normalized to 1.73M^2 body surface area.

If African-American, multiply by 1.2

Test performed at:
Rapid City Medical Center
2820 Mount Rushmore Road
Rapid City, SD 57701 (605)721-8173

RCMC Pain Medicine
101 E MINNESOTA ST RAPID CITY, SD 57701-7756
l: 605-342-3280 Fax: 605-721-8439

Tyler Ptacek MD
Pain Medicine

tient Information: **01/12/2026**

Name:	Pulliam, Viktoriya
DOB:	12/06/1989, 36Y, Sex: Female
Address:	[illegible]
Home Phone:	[illegible]
Cell Phone:	50[illegible]

Order Information:

Order Date:	10/06/2025
Assessments:	Hemiparesis, unspecified hemiparesis etiology, unspecified laterality - G81.90
DI:	**MRI Lumbar without contrast (72148)**
Fasting:	No
Specimen:	Collection Date: 10/16/2025 Time:
Clinical Info:	

Name	Value	Reference Range
Result:		

Received Date: 10/17/2025

Notes:
Campbell County Hospital, Gillette WY; needs to have complete before pump trial before 10/23
Fletcher, Norika K 10/10/2025 09:39:28 AM MDT > Unable to complete via Evicore portal as unable to find facility. Called Evicore at 888-622-7329 and spoke with Michelle, initiated case over the phone, ref# 1250083839. Case went to eligiblity review because of the facility, can take up to 24hrs.
Fletcher, Norika K 10/13/2025 02:29:35 PM MDT > Uploaded clinicals to Evicore, case # 1250083839
Fletcher, Norika K 10/14/2025 04:21:35 PM MDT > This is approved, valid 10/10/25-4/11/26. Auth# A256031024
Slone, Amanda M 10/15/2025 07:14:27 AM MDT > faxed

Patient Name: Pulliam, Viktoriya, 36Y, F, DOB: 12/06/1989

Campbell County Health

Pulliam, Viktoriya A
10/16/2025 Magnetic Resonance Imaging (MRI)
MRN: 10132149

Description: **Female DOB: 12/6/1989**
Provider: **CCH MRI OP CTR**
Department: **CCH RAD MRI SCAN OP CTR**

Dept Phone: **307-688-1600**
Encounter #: **443619274**

MR LUMBAR SPINE WITHOUT CONTRAST Final result Order: 1226145690

Status: **Final result**
Test Result Released: **Yes (not seen)**
Details

Reading Physician	Reading Date	Result Priority
Rigsby, Paul, MD 307-688-1600	10/16/2025	

Narrative & Impression
EXAMINATION: MRI LUMBAR SPINE WITHOUT CONTRAST
STUDY TYPE: MR LUMBAR SPINE WITHOUT CONTRAST
AGE, SEX: 35y, female
HISTORY: Hemiparesis, unspecified hemiparesis etiology, unspecified laterality.
COMPARISON: CT abdomen pelvis from August 14, 2024

Technique: Noncontrast multiplanar multisequence imaging of the lumbar spine.

FINDINGS:

9 mm cyst redemonstrated at the anterior left renal cortex.

Alignment is maintained. Moderate disc space narrowing at L4-L5 with mild associated degenerative endplate changes (including type 1 Modic). Remaining disc spaces and vertebral body heights appear preserved. No suspicious focal marrow signal changes are identified. Visualized lower thoracic cord appears normal in caliber without definite signal abnormality. Conus medullaris terminates near T12-L1.

L1-L2: No focal disc abnormality, significant central canal stenosis or neural foraminal narrowing.

L2-3: No focal disc abnormality, significant central canal stenosis or neural foraminal narrowing.

L3-L4: No focal disc abnormally, significant central canal stenosis or neural foraminal narrowing.

L4-L5: Disc bulge which partially effaces the ventral thecal sac and is in close proximity to the exiting bilateral L4 nerve roots. Mild central canal stenosis. Mild bilateral neural foraminal narrowing.

L5-S1: No focal disc abnormality, significant central canal stenosis or neural foraminal narrowing.

IMPRESSION:

1. Disc bulge at L4-5 which contributes to mild central canal stenosis and mild bilateral neural foraminal narrowing.
2. Small left renal cyst.
-Electronically Signed By: Paul Rigsby
-Electronically Signed On: 10/16/2025 3:59 PM

Pulliam, Viktoriya A (MRN 10132149) Printed by [EDIRADIN] at 10/16/2025 4:01 PM

Result Care Coordination

Patient Communication

Released Not seen

Associated Diagnoses

Hemiparesis, acute (HC CODE)

Comments

Order Questions

Question	Answer
Is this a professional/elite athlete requiring verbal discussion prior to report finalization?	**No**
Info the radiologist should know (e.g. Non smoker with cough for 3 weeks. Concern for pneumonia):	**Hemiparesis, unspecified hemiparesis etiology, unspecified laterality**
Ok to be altered per Radiologist-approved protocols	**Yes**
Will the patient need anxiolytic medication for this exam?	**Don't Know**
Note: * Please ensure the patient has had a documented H&P within the last 30 days.	
Is this radiologic exam going to be billed to a Research Grant/Study? If you do not know, contact the research coordinator.	**No**
Preferred Method of Communication:	**Secure Chat**
Does the patient have any implanted devices such as a pacemaker, ICD, stimulator, aneurysm clips, or any metallic foreign body?	**Don't Know**
Which region/location is the preferred scheduling location? (Use SCHED INST: field to add specific location info)	**Campbell County Health**

Note: This question is used to indicate which region will schedule the order. Continue to use the Scheduling Instructions field if you need to specify a specific hospital/clinic.

Provider Information

Ordering User	Ordering Provider	Authorizing Provider
Parkin, Kayla	System, Provider Not In	Ptacek, Tyler Anthony, MD
PCP	Billing Provider	
Sayed Ahmed, Ahmed Sayed Ahmed Mohamed, MD	Rigsby, Paul, MD	

Specimen Information

Specimen ID: 45625579
Collected: 10/16/2025 3:25 PM - 3:25 PM

Lab Information

Lab
CCH PACS POWERSCRIBE
No contact information on file